

Classic Kaposi Sarcoma

EPIDEMIOLOGY

The annual incidence rates for classic Kaposi sarcoma (KS) vary significantly across geographic regions, ranging from 0.14 per million inhabitants in Great Britain to 10.5 per million in men and 2.7 per million in women in Italy. A marked male predominance has been consistently observed since the initial descriptions of the disease, with a male-to-female ratio of approximately 3:1 in most large studies. The higher incidence in countries bordering the Mediterranean Sea has led some researchers to propose a "Mediterranean variant" of KS as a distinct entity. However, due to similarities in clinical course and behavior, this variant is not considered separately from classic KS.

CLINICAL MANIFESTATIONS

The typical patient with classic KS is a Caucasian male in his sixth decade of life, often with Mediterranean or Jewish ancestry. Lesions usually begin on the skin of the distal lower extremities—particularly the dorsum of the feet—as unilateral or bilateral bluish-red macules that may resemble hematomas. These lesions progress slowly, both horizontally and vertically, evolving into firm plaques and eventually into nodules. Over time, the color of the lesions changes to brown, and the overlying skin becomes hyperkeratotic. Ulceration is common, especially on the lower limbs. Perilesional pitting edema is frequently present and may progress to fibrosis.

Classic KS generally follows an indolent course, with gradual proximal extension over years or even decades. Many patients live with slowly progressive disease for extended periods. After several years, visceral dissemination may occur, with involvement of lymph nodes, mucous membranes, and internal organs—particularly the gastrointestinal tract—though gastrointestinal lesions are often asymptomatic. Given the advanced age of most affected individuals, death from unrelated causes frequently precedes widespread disease progression.

African Endemic Kaposi Sarcoma

EPIDEMIOLOGY

An increased incidence of Kaposi sarcoma among Black Africans in Central Africa was recognized decades before the emergence of the AIDS pandemic. In some countries in this region, KS accounts for up to 10% of all malignancies in men. There is a strong male predominance, with male-to-female ratios ranging from 3:1 in children to as high as 18:1 in adults. The age at onset is lower than in classic KS, with significant presentation in younger adults and even children.